

إسم المريض رابعياً : نهى محمد عبدالله فولى

المحافظة : القاهرة

الحى / المدينة : مصر الجديده

**العنوان : 21 ش فريد سمبكة
مصر الجديدة**

تاريخ تسجيل الإستمارة : 23-09-2026

رقم البطاقة : 27810230100385 رقم التليفون : 01001918203

العمر (تاريخ الميلاد) : 23-10-1978 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (-----)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودورنيك [14157]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

ختم

شعار الجمهورية

--

Name: نهى محمد عبدالله فول National ID: 27810230100385	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الاستشارة
طبية

03/11/2026

MDT FORM

Name:	Noha Mohamed Abd Allah .			MRN:	1082
Age:	47 Y	ID:	27810230100385		
Menopausal Sta	PRE	Phone:	01001718203		
NRS SCORE:		Marital Status:	MARRIED		
BMT:		Family History:	+VE FH		
CO Morbidity:	NO	Reason Of MDT:			

Clinical Data:

BX: IDC GII

ER: 4/8 **PR:** 3/8 **HER2:** +1 **KI67:**

R

L

Sonomamimography:

Mets Workup:

History:

Diagnosed with Lt breast cancer in 2017, for which she underwent Lt MRM followed by adjuvant chemotherapy (AC x6, then Docetaxel x6) and PORT. She was maintained on Tamoxifen for 6 years.

In 2024, the patient developed metastatic disease to bone and received Palbociclib + Fulvestrant for 3 cycles with mixed response + ZOMETA + PALLIATIVE RTH ON RT PELVIS, then shifted to Ribociclib + Fulvestrant for 3 cycles, with disease progression. She was subsequently started on Capecitabine for 3 months, but showed further progression involving bone, lymph nodes, lungs, and pericardial effusion.

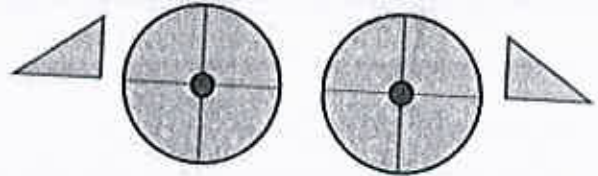
The patient then received Paclitaxel + Carboplatin for 12 weeks (3 months)

Recommendations:

CONTINUE ANOTHER 6 WEEKS OF TAXOL , CARBO WITH CLOSE FU OF PN

MDT Outcome & Plan

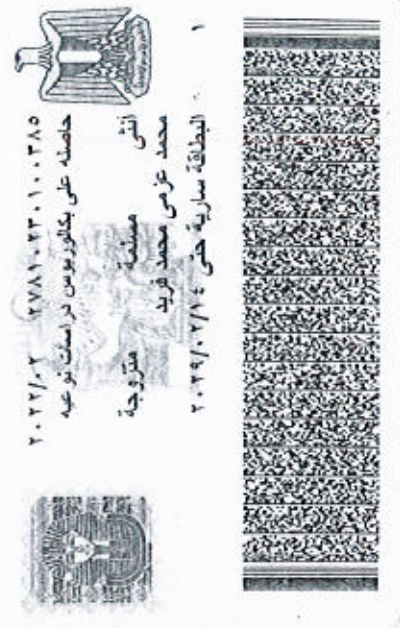
CONTINUE ANOTHER 6 WEEKS OF TAXOL weekly , CARBO AUC=5 q 3 wks with GCSF support...WITH CLOSE FU OF PN + zometa 3 months



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين	د/ اميره عديوي				



48



MDT FORM

20 / 10 / 2025

G

e	MRN	6702 / 0.00
	ID	0711.02.1.0.000
	Phone	01001918203
opausal status	Marital status	married
SCORE	Family history	+ve (mother breast ca)
	REASON OF MDT	
comorbidity		

Clinical Data

④ → Metastatic adenocarcinoma (primary site not known)				 TNM M. loc
4/8	PR: 3/3	HER 2: 1+	KI67:	
mammography:				
s Workup:				

Recommendations:

16 1st MDT → adjuvant CA → PORT → 6 x 7 yrs
2024 while on adjuvant CA, pt developed bone mets
→ 1st line CAK 4/65 @ AI x 6mths → DP → 2nd line
Xeloda with initial PR → PR/CA 10/12/21 (NO)

OUTCOME & PLAN

BRCA -ve

NOT to start Carboplatin & paclitaxel (weakly
Aug-2025)

SIGNATURE

DICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
20/10/2025					

المراسلة : 27810230100385

: 27810230100385

: SR-1165/24

: Female

: 45 Y

Referring Doc :

Request No : 13001961

Request Date : 15/08/2024

Verification Date : 26/08/2024

Financial Category : Out Patient

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical data:

L1 osteolytic lesion in a known case of breast cancer.

Nature of specimen:

Slide and block revision.

Gross examination

One block labelled by (871297) and seven slides were received.

Microscopic examination:

Sections examined revealed cores of musculoskeletal and fatty tissue infiltrated by malignant epithelial cells of moderate pleomorphism and hyperchromatic nuclei arranged in sheets and acini.

-Examined smears slides were positive for malignancy.

PT:

ER: Moderate nuclear reaction in 5% of tumor cells (score 4/8).

PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).

Her2: Weak incomplete membrane reaction in >10% of tumor cells (score 1+).

Diagnosis:

1 osteolytic lesion, slide and block revision:

Metastatic adenocarcinoma, Grade II of primary mammary ductal origin.

Cytology: Positive for malignancy.

T:

R: Positive (score 4/8).

R: Positive (score 3/8).

er2: (score 1+).

Verified by :

dr.esraa magdy

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Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

ا.د. أحمد كمال الدري
 ا.د. محمد أبو الهدى درويش
 ا.د. عمر حسين عمر
 ا.د. وحيد حسين طنطاوي
 ا.د. ياسر عبد العظيم عباس
 ا.د. أحمد محمد منيب
 أساتذة الأشعة التشخيصية
 كلية الطب - جامعة عين شمس

Patient Name: Noha Mohamed Abd Allah	Study Date: 15/07/2024
Ref. Phy.: طبيب امكا	Patient ID: 1128069

TRANSVAGINAL PELVIC ULTRASONOGRAPHY:-

FINDINGS:-

- Uterus AVF, of normal size, measuring 10 x 6 x 7 cm. (in maximum longitudinal X A.P. X Transverse diameter).
- Cervix: homogeneous
- myometrium is showing anterior mid corporeal partial subserous/ interstitial myoma 10 mm FIGO 5.
- Endometrial lining of 9 mm in thickness, with echogenic foci, yet no intra-cavitary lesions.
- Both Ovaries: both are enlarged with a simple cyst at each side (Rt = 4 x 3.7 cm, Lt = 4.5 x 3.5 cm), no internal vascularity or solid components. The right cyst shows incomplete internal septation, with rather oblong shape.
- No pelvic masses or cysts.
- Minimal free pelvic fluid collection.

IMPRESSION:-

- Uterine myoma FIGO 5.
- Bilateral ovarian simple cysts, the right may representing hydrosalpinx, for follow up.

MUCH OBLIGED
Dr.Nadin Fawzy.MD

د. أحمد كمال الدري
د. محمد أبو الهدى مريش
د. عمر حسين عبد
د. وحيد حسين طناوي
د. ياسر عبد العظيم عباس
د. أحمد محمد مريب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Noha Mohamed Abd Allah	Patient ID: 1128069
Prof. Dr. Shimaa Lasheen	Study Date: 11/10/2025
Acc No: 2413886	

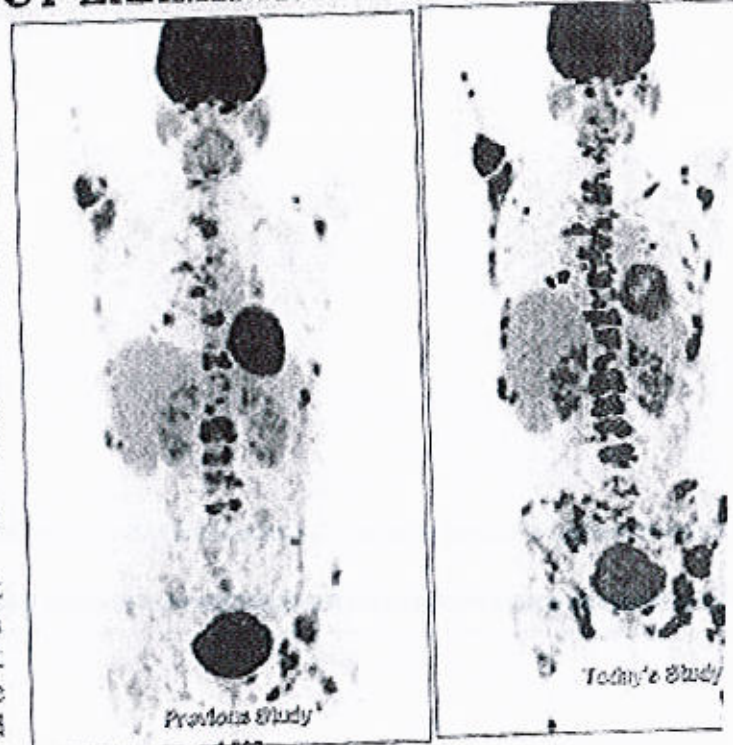
F18- FDG PET/CT EXAMINATION

History:-

A 47-years-old female patient, presented with history of metastatic left breast cancer, underwent left mastectomy in 2017 as well as received chemotherapy, radiotherapy, targeted therapy and now on hormonal treatment. Referred for follow up PET/CT examination.

Procedure:-

- 6 mCi ¹⁸F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 89 mg/dl.
- A diagnostic 128 MDCT post contrast examination was taken after I.V. non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.



PET/CT FINDINGS:-

Compared to previously done PET/CT study dated 30/6/2025, the current study revealed:

A) Head and Neck:-

- Still no hypermetabolic cervical lymph nodes detected.
- The visualized portions of the brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.

د. أحمد
د. محمد أبو
د. محمد حسن
د. وحيد حسين
د. ياسر عبد العظيم
د. أحمد محمد منيب

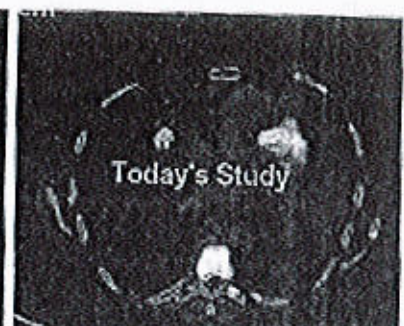
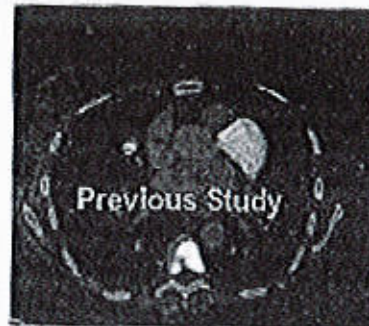
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Noha Mohamed Abd Allah	Patient ID: 1128069
Prof. Dr. Shimaa Lasheen	Study Date: 11/10/2025
Acc No: 2413886	

- No FDG-avid parenchymal thyroid gland focal lesions could be elicited.

B) Chest:-

- The left mastectomy operative bed is still clear and devoid of any hypermetabolic soft tissue lesions that would account for recurrent neoplasia.
- Clear right breast parenchyma with no definite sizeable hypermetabolic soft tissue masses detected.
- **Metabolic and morphologic progression** of the previously detected irregular hypermetabolic pleural based right lung middle lobar paracardiac nodule, currently measuring 2.2 x 1.9 cm and achieving up to 14.7 SUV max (compared to 2cm and achieving 8.6 SUV max previously) with related fibrotic bands.
- No newly developed other metabolically active pulmonary nodules detected.
- Small insignificantly avid bilateral hilar and gastroesophageal/aorto-esophageal lymph nodes, measuring up to 13.3mm and achieving up to 4.94 SUV max (compared to 4.5 SUV max previously).
- No pleural effusion.
- Still noted the mild pericardial effusion.
- Normal tracer biodistribution along the myocardium is noted.
- Still noted small sliding hiatus hernia.



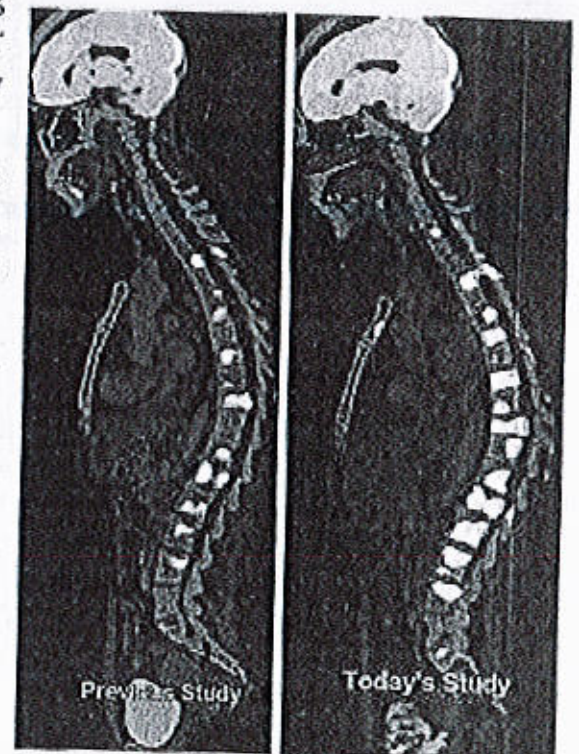
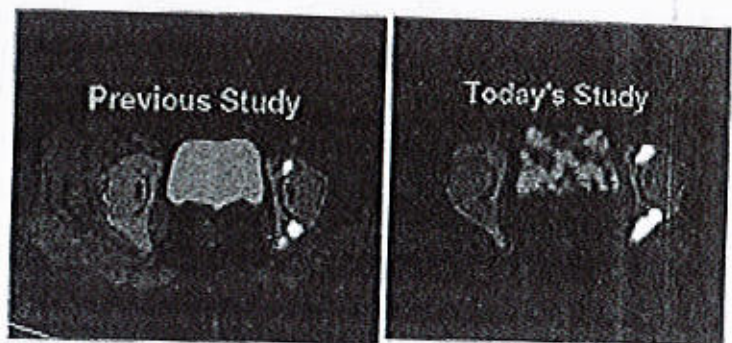
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أ.د. ياسر عبد العظيم
أ.د. أحمد محمد منير

أستاذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Noha Mohamed Abd Allah	Patient ID: 1128069
Prof. Dr. Shimaa Lasheen	Study Date: 11/10/2025
Acc No: 2413886	

C) Abdomen & Pelvis:-

- The liver is still of average size with no FDG avid parenchymal focal lesions detected. Still noted the few small ill-defined hypodense inactive hepatic focal lesions at segments V and VI, measuring up to 14 mm, showing peripheral irregular nodular enhancement currently, likely hemangiomas.
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Physiological uptake in adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.



D) Musculoskeletal:-

- **Overall progressive course** of the previously detected hypermetabolic osseous lesions seen randomly distributed along the with axial and appendicular skeleton the with newly developed others, the most active one at right humeral head achieving 19.33 (compared to 15.1 SUV max previously).

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أ.د. ياسر عبد العظيم عباس
أ.د. أحمد محمد منيب

أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

Patient Name: Noha Mohamed Abd Allah	Patient ID: 1128069
Prof. Dr. Shimaa Lasheen	Study Date: 11/10/2025
Acc No: 2413886	

CONCLUSION:-

Follow up PET/CT study revealed:

- Still no evidence of recurrent breast neoplasia.
- Metabolic and morphologic progression of the metastatic right lung middle lobar pulmonary nodule.
- Few insignificantly avid bilateral hilar and mediastinal gastroesophageal/aorto-esophageal lymph nodes for follow-up.
- Overall progressive course of the metastatic osseous deposits.
- Further follow-up is still recommended to monitor therapeutic response.

MUCH OBLIGED,
HANY RAAFAT, MD

HEBA SAMIR, MD

د. أحمد جمال الدري
د. محمد أبو الهدي درويش
د. عمر حسين عمر
د. وحيد حسين طنطاوي
د. ياسر عبد العظيم عباس
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أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS: NOHA MOHAMED ABD ALLAH
PROF DR. SHAIMAA LASHIN

PATIENT ID: 1128069
STUDY DATE: 19/09/2026

F18- FDG PET/CT EXAMINATION

History:-

A 47-year-old female patient, presented with history of metastatic left breast cancer, underwent left mastectomy in 2017 as well as received chemotherapy, targeted therapy and hormonal treatment. Referred for further follow up PET/CT examination.

Procedure:-

- 6.6 mCi 18F-FDG was administrated to the patient and imaged after appropriate time.
- The blood glucose level was 90 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- A diagnostic 128 MDCT non-contrast examination. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:-

Compared to previously done PET/CT report dated 15/06/2026, the current study revealed:

A) Head and Neck:-

- The neck is devoid of any abnormally hypermetabolic lymph nodes.
- The brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- No FDG avid parenchymal thyroid gland focal lesions could be elicited.

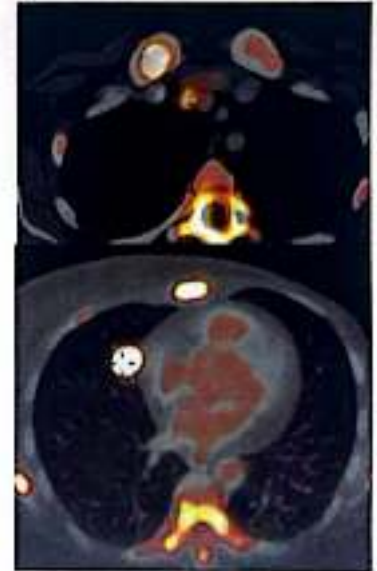


MRS: NOHA MOHAMED ABD ALLAH
PROF DR. SHAIMAA LASHIN

PATIENT ID: 1128069
STUDY DATE: 19/09/2026

B] Chest:-

- The left mastectomy operative bed as well as the right breast are devoid of any FDG avid soft tissue density lesions that may account for recurrent neoplasia.
- **Newly developed small metabolically active, enlarged superior mediastinal/pre-tracheal lymph node** is noted measuring +/- 12x9mm and achieving 6.15 SUVmax.
- Few small insignificantly avid right axillary lymph nodes are noted achieving up to 3.8 SUVmax, likely reactive for follow up.
- **Almost stationary course** of the previously reported **irregular right middle lobar paracardiac lung nodule, based on the mediastinal pleura**, currently measuring +/-17x21mm and achieving 16.47 SUVmax (compared to 15X20 mm and 15.7 SUVmax previously).
- No pleural effusion detected.
- Normal tracer biodistribution along the myocardium is noted. Minimal pericardial effusion.
- In situ right Port-A-Cath is noted currently.
- Still noted axial hiatus hernia.



C] Abdomen & Pelvis:-

- Still noted the average sized liver with ill-defined inactive subcapsular focal lesions at segments V and VI showing peripheral nodular enhancement, likely representing hemangiomas, still measuring up to +/-14 mm.
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Physiological uptake in adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.

MUCH OBLIGED
DR. HEBA SAMIR MD
PROF. DR. OMAR HUSSEIN

MUCH OBLIGED

- Follow up PET/CT study revealed:
- Almost stationary course of the active right middle lobar lung nodule.
 - Over all regressive course of the osseous metastatic deposits.
 - Newly developed small metabolically active superior mediastinal/pre-tracheal lymph node, for follow up
 - No recurrent breast neoplasia.

CONTO: NOIS-

Over all regressive course of the metabolically active osseous lesions that are infiltrating the axial skeleton as well as humeral and femoral bones, currently increasing up to 15.2 SUVmax at proximal right humeral shaft (compared to 22.8 SUVmax previously).

DI Musculoskeletal:-

MRS: NOHA MOHAMED ABD ALLAH
PROF DR. SHAIMAA LASHIN

PATIENT ID: 1128069
STUDY DATE: 19/09/2026

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